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DIAGNOSTIC IMAGING REPORT

PATIENT	MARQUEZ, Linda	DOB / AGE / SEX	03/22/1968 / 58 / Female
MRN	00482917	ACCESSION	BRI-26-0081734
ORDERING PROVIDER	Rebecca Lindqvist, PA-C, Riverside Urgent Care	EXAM DATE	10/14/2026 02:05 PM
INSURANCE	Bluebonnet Health, Bluebonnet PPO 500	MEMBER ID	BBH-4471-0932
REPORT STATUS	Final	REPORTED	10/14/2026 04:52 PM

EXAM:

CT CHEST WITHOUT CONTRAST

CLINICAL INDICATION:

Persistent cough for 6 weeks, not improved after a course of antibiotics. Former smoker (approximately 20 pack-years, quit 2016). Evaluate for pneumonia or other cause.

TECHNIQUE:

Helical CT images of the chest were obtained from the thoracic inlet through the upper abdomen without intravenous contrast. Axial images were reconstructed with coronal and sagittal reformats. Automated exposure control and iterative reconstruction were used to minimize radiation dose. CTDIvol: 6.2 mGy. DLP: 241 mGy-cm.

COMPARISON:

Chest radiograph dated 09/30/2026 (outside study, report only).

FINDINGS:

Lungs and airways: Mild bronchial wall thickening in both lower lobes. No consolidation, ground-glass opacity, or mucus plugging. There is a 9 mm solid noncalcified nodule in the superior segment of the right lower lobe (series 3, image 142) with smooth margins. No other suspicious pulmonary nodules. Mild centrilobular emphysema, upper lobe predominant. The central airways are patent.

Pleura: No pleural effusion or pneumothorax.

Heart and mediastinum: No mediastinal or hilar lymphadenopathy by size criteria. The heart is normal in size. No pericardial effusion. Mild coronary artery calcification in the left anterior descending artery. The thoracic aorta is normal in caliber.

Lower neck: 18 mm hypodense nodule in the left lobe of the thyroid gland (series 2, image 18).

Upper abdomen: Limited evaluation without contrast. The visualized liver, spleen, adrenal glands, and upper kidneys are unremarkable.

Bones and soft tissues: Mild degenerative changes of the thoracic spine. No suspicious osseous lesion.

IMPRESSION:

1. Mild bronchial wall thickening in the lower lobes, which can be seen with bronchitis or reactive airways disease. No pneumonia.
2. 9 mm solid noncalcified nodule in the superior segment of the right lower lobe. For a solid nodule greater than 8 mm, Fleischner Society 2017 guidelines suggest considering CT at 3 months, PET/CT, or tissue sampling.
3. 18 mm left thyroid nodule. Recommend thyroid ultrasound if not previously evaluated.
4. Mild centrilobular emphysema.

Electronically signed by: Daniel Okafor, MD
Signed: 10/14/2026 04:52 PM
Interpreting facility: Barton Ridge Imaging Center

SYNTHETIC TEST DOCUMENT. Fictional patient. Hackathon prototype, not medical advice.